Referral packet F01: readable copy

Source: F01.pdf (fictional), 11 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

Page map

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1	Fax cover sheet	fax_cover	good	
2	IHH Recuperative Care Referral Form	referral_form	good	
3	CalAIM Community Supports authorization request	calaim_auth	good	
4	Patient face sheet	face_sheet	good	
5	History and Physical - page 1	h_and_p	good	
6	History and Physical - page 2	h_and_p	good	
7	Wound care nurse note	wound_note	good	
8	Hospitalist progress note and CM note	progress_note	good	
9	Vital signs flowsheet	vitals	good	
10	Laboratory results	labs	good	
11	Discharge medication list	med_list	good	



FAX F01 FICTIONAL DEMO RECORD - NOT A REAL PATIENT PAGE 01/11

Fax cover sheet

Harbor View Medical Center (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Janet Okafor, RN CM
Phone: 555-0110
Date: 03/05/2026 09:20
Pages (including cover): 11
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
Referral for recuperative care for 46F, LLE cellulitis, improving on PO abx. Anticipated d/c 03/05 PM. Home health (Coastline Home Health) SOC 03/06 for wound checks.
Please call CM with questions.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictiona..

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Harbor View Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Janet Okafor, RN CM
Phone: 555-0110
Date: 03/05/2026 09:20
Pages (including cover): 11
Re: Recuperative care referral
Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

Referral for recuperative care for 46F, LLE cellulitis, improving on PO abx. Anticipated d/c 03/05 PM. Home health (Coastline Home Health) SOC 03/06 for wound checks.
Please call CM with questions.

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-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Harbor View Medical Center (fictional) Dept/Floor: 4 West Med-Surg
SW/CM/RN: Janet Okafor, RN CM Phone/Pager #: 555-0110
Email: jokafor@harborview.example Nursing Station #: 555-0111
Authorized by: Dr. P. Lindqvist Phone #:

Patient Information

Patient Name: Rosa Doe MRN: HV-0048213 Date of Birth (dd/mm/yyy): 08/14/1979
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Hispanic/Latina
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Cellulitis L lower leg
Admit Date: 03/02/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☒ Yes ☐ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 03/02/2026 Results: CXR neg - no acute process

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ✕No	Active Infection	✕Yes* No
*Explain		*Explain cellulitis on PO abx	
Self Administer All Meds	✕Yes No*	Ambulatory	✕Yes No
Continent of Bowel & Bladder	✕Yes No	Assistive Device	Yes* ✕No
Colostomy/Ileostomy	Yes ✕No	*Explain	
Foley Catheter	Yes ✕No	Hospice	Yes* ✕No
Diabetes	Yes ✕No	*Explain	
Insulin	Yes ✕No	Car	Yes ✕No
Diabetes related Oral Meds	Yes ✕No	Service Animal	Yes ✕No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ✕No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain none

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Harbor View Medical Center (fictional)
Department/Floor: 4 West Med-Surg
Social Worker/Case Manager/Nurse: Janet Okafor, RN CM
Phone: 555-0110
Email: jokafor@harborview.example
Nursing Station: 555-0111
Authorized By: Dr. P. Lindqvist

Patient Information

Name: Rosa Doe
MRN: HV-0048213
Birth Date: 08/14/1979
Gender: F
Ethnicity: Hispanic/Latina
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Cellulitis L lower leg
Admit Date: 03/02/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes **[X]** No [ ]
Home Health Ordered: Yes **[X]** No [ ]
PPD/TB Test or Chest X-Ray: Date: 03/02/2026 Result: CXR neg - no acute process

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes **[X]** No [ ] cellulitis on PO abx
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other: none

Length of Stay Authorized

Days: 14

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

-- FICTIONAL DEMO RECORD - not a real patient --

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Rosa Doe
DOB: 08/14/1979
Member ID: OCH-77120456
Referring Provider
Facility: Harbor View Medical Center (fictional)
Clinical
Primary Diagnosis: Cellulitis of left lower limb
ICD-10: L03.116
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 03/05/2026
Requested Days: 14
Clinical summary: Cellulitis LLE improving on PO antibiotics. Independent ADLs.
Plan Use Only
Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Rosa Doe
DOB: 08/14/1979
Member ID: OCH-77120456

Referring Provider

Facility: Harbor View Medical Center (fictional)

Clinical

Primary Diagnosis: Cellulitis of left lower limb
ICD-10: L03.116
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 03/05/2026
Requested Days: 14
Clinical summary: Cellulitis LLE improving on PO antibiotics. Independent ADLs.

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Harbor View Medical Center (fictional) - PATIENT FACE SHEET
Patient: Rosa Doe
MRN: HV-0048213
Account #: 99048213
DOB: 08/14/1979
Sex: Female
Preferred Language: Spanish / English (bilingual)
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 03/02/2026 16:48
Admit Type: Inpatient
Unit/Bed: 4W-412B
Attending: Priya Lindqvist, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Harbor View Medical Center (fictional) - PATIENT FACE SHEET

Patient: Rosa Doe
MRN: HV-0048213
Account #: 99048213
DOB: 08/14/1979
Sex: Female
Preferred Language: Spanish / English (bilingual)
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 03/02/2026 16:48
Admit Type: Inpatient
Unit/Bed: 4W-412B
Attending: Priya Lindqvist, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL
Date of Service: 03/02/2026
Attending: Priya Lindqvist, MD (Hospital Medicine)
Chief Complaint
"My left leg is red and swollen" x 4 days
HPI
46 y/o F experiencing homelessness (sleeps in vehicle-less encampment near riverbed) with no significant PMH presents with 4 days of progressive erythema, warmth and pain of the L anterior shin after a scrape on a fence. Subjective fevers/chills x 2 days. Denies IVDU. No prior MRSA infection known. In ED: T 101.2F, HR 108, WBC 14.2. Received vancomycin 1.5 g IV x1 and cefazolin 2 g IV.
PMH
- Seasonal allergies
- No DM, no HTN
PSH
- C-section 2008
Social History
Homeless x 18 months. Tobacco 1/2 ppd x 20 yrs. EtOH: social, last drink 1 wk ago. Denies drug use.
Allergies
NKDA
Home Medications
None

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date of Service: 03/02/2026
Attending: Priya Lindqvist, MD (Hospital Medicine)

Chief Complaint

"My left leg is red and swollen" x 4 days

HPI

46 y/o F experiencing homelessness (sleeps in vehicle-less encampment near riverbed) with no significant PMH presents with 4 days of progressive erythema, warmth and pain of the L anterior shin after a scrape on a fence. Subjective fevers/chills x 2 days. Denies IVDU. No prior MRSA infection known. In ED: T 101.2F, HR 108, WBC 14.2. Received vancomycin 1.5 g IV x1 and cefazolin 2 g IV.

PMH

- Seasonal allergies
- No DM, no HTN

PSH

- C-section 2008

Social History

Homeless x 18 months. Tobacco 1/2 ppd x 20 yrs. EtOH: social, last drink 1 wk ago. Denies drug use.

Allergies

NKDA

Home Medications

None

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Physical Exam
VS: T 100.8F, HR 102, BP 128/76, RR 18, SpO2 98% RA
Gen: NAD, AOx4
CV: RRR, no m/r/g
Lungs: CTAB
Skin: L anterior shin erythema approx 14 x 9 cm, marked with pen, warm, tender. Central superficial open area 2.0 x 1.5 cm, minimal serous drainage, no fluctuance. No crepitus.
Ext: 1+ edema LLE, DP pulses 2+ bilat
Neuro: grossly intact
Assessment / Plan
46F with non-purulent cellulitis L lower leg with small superficial ulceration, mild SIRS on arrival.
1. Cellulitis LLE - IV cefazolin 2 g q8h; LE venous duplex to r/o DVT; elevate; wound care consult; blood cx x2
2. MRSA nares PCR, COVID per protocol
3. Tobacco use - nicotine patch 14 mg offered
4. Dispo - CM consult; homeless, anticipate recuperative care referral
DVT ppx: enoxaparin 40 mg SC daily
Code status: Full
Electronically signed: Priya Lindqvist, MD 03/02/2026 21:14

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 6: History and Physical - page 2

h_and_p | orientation normal | legibility good

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DVT ppx: enoxaparin 40 mg SC daily
Code status: Full
Electronically signed: Priya Lindqvist, MD 03/02/2026 21:14

-- FICTIONAL DEMO RECORD - not a real patient --

Wound care nurse note

WOUND CARE CONSULT NOTE
Date: 03/04/2026 11:05
Wound #1: L anterior shin, superficial open area within cellulitis
Size: 1.8 cm L x 1.2 cm W x 0.1 cm D (improved from 2.0 x 1.5)
Wound bed: 90% pink granulation, 10% yellow slough
Drainage: scant serous. Odor: none
Periwound: erythema receding, now inside marked borders
Treatment
- Cleanse with NS, apply hydrogel, cover with foam dressing
- Change dressing every other day and PRN
Education
Pt return-demonstrated dressing change independently x2. Verbalized s/s of worsening infection.
Recommendation
Home health RN for wound check 2x/week x 2 weeks.
Signed: L. Ferreira, RN CWOCN

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 7: Wound care nurse note

wound_note | orientation normal | legibility good

WOUND CARE CONSULT NOTE

Date: 03/04/2026 11:05
Wound #1: L anterior shin, superficial open area within cellulitis
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Education

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Recommendation

Home health RN for wound check 2x/week x 2 weeks.
Signed: L. Ferreira, RN CWOCN

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note and CM note

HOSPITALIST PROGRESS NOTE - HD#4
Date: 03/05/2026 07:40
S: Pain 2/10, ambulating halls independently. No fevers x 48h.
O: T 98.4F HR 78 BP 122/74 RR 16 SpO2 99% RA
Erythema markedly receding. Duplex 03/03 neg for DVT.
Blood cx 03/02 x2: no growth at 72h.
A/P: Cellulitis LLE improving.
- Transition to PO cephalexin 500 mg QID + TMP-SMX DS 1 tab BID to complete 7 days total
- Medically stable for d/c to recuperative care today
CASE MANAGEMENT NOTE
Date: 03/05/2026 08:55
Pt AOx4, independent with ADLs and ambulation (no device). Pt agreeable to recuperative care.
Home health referral accepted by Coastline Home Health (fictional), 555-0131; RN SOC visit scheduled 03/06/2026 for wound check, then 2x/week.
Abx script to be filled at bedside pharmacy (30-day supply policy n/a; 7-day course).
Signed: Janet Okafor, RN CM

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 8: Hospitalist progress note and CM note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#4

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Abx script to be filled at bedside pharmacy (30-day supply policy n/a; 7-day course).
Signed: Janet Okafor, RN CM

-- FICTIONAL DEMO RECORD - not a real patient --

Vital signs flowsheet

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
03/03 20:00	99.1	88	126/78	18	98%	RA
03/04 08:00	98.6	84	124/76	16	99%	RA
03/04 16:00	98.7	80	120/72	16	98%	RA
03/05 00:00	98.2	76	118/70	16	99%	RA
03/05 07:40	98.4	78	122/74	16	99%	RA

Pain 2/10 L shin. Ambulating independently.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 9: Vital signs flowsheet

vitals | orientation normal | legibility good

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
03/03 20:00 | 99.1 | 88 | 126/78 | 18 | 98% | RA
03/04 08:00 | 98.6 | 84 | 124/76 | 16 | 99% | RA
03/04 16:00 | 98.7 | 80 | 120/72 | 16 | 98% | RA
03/05 00:00 | 98.2 | 76 | 118/70 | 16 | 99% | RA
03/05 07:40 | 98.4 | 78 | 122/74 | 16 | 99% | RA
Pain 2/10 L shin. Ambulating independently.

-- FICTIONAL DEMO RECORD - not a real patient --

Laboratory results

LABORATORY RESULTS
Collected | Test | Result | Ref
03/02 14:30 | WBC | 14.2 H | 4.0-11.0 K/uL
03/04 05:00 | WBC | 9.1 | 4.0-11.0 K/uL
03/04 05:00 | Hgb | 12.8 | 12.0-16.0 g/dL
03/04 05:00 | Plt | 288 | 150-400 K/uL
03/04 05:00 | Creatinine | 0.7 | 0.5-1.1 mg/dL
03/04 05:00 | Glucose | 96 | 70-99 mg/dL
03/02 14:30 | Lactate | 1.4 | 0.5-2.0 mmol/L
MICROBIOLOGY
03/02 Blood culture x2: No growth at 72 hours (final pending 5 days)
03/03 06:15 MRSA nares PCR: NEGATIVE
03/05 06:10 SARS-CoV-2 rapid antigen: NEGATIVE
IMAGING
03/02 CXR PA/Lat: No acute cardiopulmonary process
03/03 LLE venous duplex: No DVT

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 10: Laboratory results

labs | orientation normal | legibility good

LABORATORY RESULTS

Collected | Test | Result | Ref
03/02 14:30 | WBC | 14.2 H | 4.0-11.0 K/uL
03/04 05:00 | WBC | 9.1 | 4.0-11.0 K/uL
03/04 05:00 | Hgb | 12.8 | 12.0-16.0 g/dL
03/04 05:00 | Plt | 288 | 150-400 K/uL
03/04 05:00 | Creatinine | 0.7 | 0.5-1.1 mg/dL
03/04 05:00 | Glucose | 96 | 70-99 mg/dL
03/02 14:30 | Lactate | 1.4 | 0.5-2.0 mmol/L

MICROBIOLOGY

03/02 Blood culture x2: No growth at 72 hours (final pending 5 days)
03/03 06:15 MRSA nares PCR: NEGATIVE
03/05 06:10 SARS-CoV-2 rapid antigen: NEGATIVE

IMAGING

03/02 CXR PA/Lat: No acute cardiopulmonary process
03/03 LLE venous duplex: No DVT

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION RECONCILIATION
Patient: Rosa Doe MRN: HV-0048213
New Medications
- Cephalexin 500 mg PO four times daily x 4 more days (last dose 03/08)
- Sulfamethoxazole-trimethoprim 800-160 mg (DS) 1 tab PO twice daily x 4 more days
- Acetaminophen 650 mg PO every 6 hours PRN pain (max 3 g/day)
- Nicotine patch 14 mg/24h, apply daily
Wound Supplies
- Hydrogel, foam dressings 4x4 (10 count), NS
Stopped
- Cefazolin IV (inpatient only)
- Enoxaparin (inpatient only)
Patient able to self-administer all medications: Yes
Reviewed with patient by: M. Tran, PharmD 03/05/2026

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional

Page 11: Discharge medication list

med_list | orientation normal | legibility good

DISCHARGE MEDICATION RECONCILIATION

Patient: Rosa Doe MRN: HV-0048213

New Medications

- Cephalexin 500 mg PO four times daily x 4 more days (last dose 03/08)
- Sulfamethoxazole-trimethoprim 800-160 mg (DS) 1 tab PO twice daily x 4 more days
- Acetaminophen 650 mg PO every 6 hours PRN pain (max 3 g/day)
- Nicotine patch 14 mg/24h, apply daily

Wound Supplies

- Hydrogel, foam dressings 4x4 (10 count), NS

Stopped

- Cefazolin IV (inpatient only)
- Enoxaparin (inpatient only)

Patient able to self-administer all medications: Yes
Reviewed with patient by: M. Tran, PharmD 03/05/2026

-- FICTIONAL DEMO RECORD - not a real patient --